

Atraumatic Headache

History

- Timing – how long has patient had headache? Sudden onset?
- Describe headache – dull, throbbing, sharp
- Does patient have a history of headaches? Is this like previous headaches?
- Other family members or persons in same vicinity of patient with similar complaints? (CO poisoning?)
- Associated symptoms – nausea/vomiting, fevers, visual disturbance, neurologic deficits, dizziness, syncope, neck stiffness, seizure, altered mental status?
- Significant medical history?
 - Pay special attention to: HTN, diabetes, past CVA, previous cranial surgery, ventriculoperitoneal shunt (VPS), eye surgery, seizures, neurologic disorders, history of head trauma, history of migraine headaches, history of sinus surgery
- New medication recently?
- Recent alcohol or drug use?
- Is patient on anti-coagulants? (Warfarin/Coumadin, Plavix, Pradaxa, Xarelto, Eliquis, aspirin, Lovenox, Aggrenox, heparin, etc.)
- Recent trauma?

Physical

- Vitals – Hypertensive? Febrile? Hyperglycemic? Hypoglycemic? Hypoxic?
- Pupil exam
- Mental status – Altered? Anxious? Agitated? Post-ictal?
- FAST Stroke scale
- VAN Stroke scale
- Signs of trauma?

Consider the possibility of:

- Stroke
- Intracranial Hemorrhage
- Meningitis
- Hypertensive crisis
- Carbon monoxide poisoning
- Seizure (new-onset)
- Acute glaucoma
- Diabetic Ketoacidosis (DKA)
- Hypoglycemia

If meningitis suspected (HA, fever, signs of sepsis, neck stiffness, AMS):

- Wear PPE
- Notify receiving hospital of possible meningitis so they can arrange for isolation room
- Contact the department's Infection Control Officer as soon as possible to report possible meningitis exposure

EMT/AEMT

- Follow General Medical Care Guideline
- Encourage fluid by mouth for hydration
- Perform a FULL neurological assessment
- Treat per Head Trauma Guideline as indicated
- Treat per Stroke Alert Guideline as indicated
- Treat per Altered Mental Status Guideline as indicated

EMT/AEMT - Expanded Scope

- **Zofran** 4mg ODT/IM every 5 min as needed x 2 doses total (8mg max)
- Pain management - see Paramedic section for guidance

Paramedic

- **NS/LR** 500 mL -1000 mL IV/IO for SBP $\leq$ 90 or other signs/symptoms of shock
- Pain management as per Pain Management Guideline - AVOID use of Toradol if maximal pain at onset (sudden onset), AMS, or neurologic deficit noted
- **Zofran** 4 mg IV/IO/IM or ODT for nausea / vomiting (may repeat x 1 in 5 min)
- **Magnesium Sulfate** 2g IV